



Exploring trauma-informed listening among public relations professionals

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ARTICLE INFO

Keywords:

Communication
Listening
Trauma
Power
Care
Public relations

ABSTRACT

There is an increasing awareness of the prevalence and impact of trauma on publics' lives, particularly in the wake of the COVID-19 pandemic. As intermediaries who shape symbols, discourses, meanings, and practices amidst evolving social, cultural, and psychological landscapes such as these, public relations professionals must remain mindful of their potential to inflict harm on the recipients of their work and understand publics' lived experiences that may involve trauma. Yet, little research has explored public relations work from a trauma-informed approach. Building upon Macnamara's (2016b, 2018) architecture of listening model, this study considers how public relations can help enact trauma-informed organizational listening. As such, this qualitative analysis of interviews with 54 public relations and strategic communication professionals yielded insights for listening with consideration for trauma and harm among diverse publics. Findings suggest that public relations professionals engage in trauma-informed listening via respect and sensitivity, listening out for invisible or unheard trauma, consideration of power distance, understanding that the trauma can be inflicted by communications work, and with consideration for 'compassion fatigue.' Combined with existing organizational listening theory, this article offers practical recommendations for how organizations can enact trauma-informed listening.

1. Introduction

Public relations professionals hold considerable power as the producers of visual, auditory, or written representations and research, implementation, and evaluation efforts for communication campaigns (Curtin & Gaither, 2007). Such work generates discourses that provide legitimacy for particular points of view, constructs social hierarchies, and produces physical and financial consequences (Edwards, 2012). In the wake of the COVID-19 pandemic, publics around the globe have experienced trauma-like and post-traumatic stress disorder-like (PTSD) symptoms (e.g., Bridgland et al., 2021; Lonsdorf, 2022). Discrimination, stress, and trauma have been particularly high among marginalized groups post-COVID-19 (e.g., Liu & Modir, 2020; Singh & Koran, 2020; Tavernise & Oppel, 2020). Public relations can also play an important role in considering the needs and experiences of internal publics, such as employees, as a result of COVID-19 (Ruck & Men, 2021). As the intermediaries who shape symbols, discourses, meanings, and practices (Edwards, 2012) amidst evolving social, cultural, and psychological landscapes such as these, public relations professionals must remain mindful of publics' lived experiences that may involve trauma. Yet, little research, however, has explored public relations work from a

trauma-informed approach (e.g. Madden & Del Rosso, 2021).

As public relations professionals seek to minimize harm and engage in culturally informed and transparent dialogue with publics, the practice of listening is central to this process. Listening with consideration of trauma may further guide ethical and compassionate public relations practice, serving as an essential component of an organizational listening and the extant 'architecture of listening' framework (Macnamara, 2016b, 2018). The purpose of this paper, therefore, is to explicate the concept of trauma-informed listening in public relations to fill the dearth of research on the topic and to build upon extant organizational listening modeling. First, the paper reviews extant literature regarding trauma-informed care, listening, the architecture of listening model (Macnamara, 2016b, 2018), and strategic communication. Next, the paper outlines the qualitative method utilized and findings generated from data analysis. Lastly, the paper concludes with a discussion of implications for trauma-informed listening and public relations theory and practice with suggestions for future research.

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<https://doi.org/10.1016/j.pubrev.2023.102393>

Received 16 January 2023; Received in revised form 9 October 2023; Accepted 9 October 2023

Available online 21 October 2023

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2. Literature review

2.1. Defining trauma and trauma-informed care

The way that scholars think about the lasting effects of trauma, both physical and psychological, has evolved greatly (Altieri & Santangelo, 2021; Arledge & Wolfson, 2001; Gerber, 2019; Hobfoll et al., 2014; Kaniasty & Norris, 1997; Poole & Greaves, 2012). As Gill (2007) explained, trauma is not only “the actual ‘blow’ to tissues of the body or structures of the mind,” but also the mental stress that often accompanies the physical trauma (p. 624). As such, trauma is not typically isolated to a single traumatic event but can be an amalgamation of a person’s lived experiences. As Erikson (1994) wrote, “trauma has to be understood as resulting from a constellation of life experiences as well as from a discrete happening, from a persisting condition, as well as from an acute event” (p. 229, italics in the original).

Trauma can also be understood at a collective level within communities impacted by war, natural disaster, or widespread violence. Events like the COVID-19 pandemic also create the conditions for collective trauma (Stanley et al., 2021), particularly within marginalized communities. Collective trauma acknowledges that psychological damage not only occurs on the individual level, but also at a community, regional, or national level (Audergon, 2004). In recent years, more research has looked at trauma on a macro scale to better understand how communities are affected by collective trauma (Audergon, 2004; Hirschberger, 2018; Somasundaram, 2014). Hirschberger (2018), for instance, studied how collective trauma then transforms into collective meaning, causing these groups to “redefine who they are and where they are going” (p. 1). In the context of the United States, collective trauma caused by supremacist prejudices has resulted in communities of people to be targeted by violence, disparity, and hatred. Furthermore, racial trauma can result from race-based stress resulting from discrimination that can have intergenerational impacts (Comas-Díaz et al., 2019). For people belonging to multiple marginalized identities, “intersectional oppression such as racial, gender, sexual orientation, and xenophobic microaggressions contribute to the cumulative effects of racial trauma” (Comas-Díaz et al., 2019, p. 1). Trauma does not just impact individuals; it also impacts communities—directly and indirectly. For example, collective trauma can present as psychological distress, transgenerational suffering, fear, and/or overexposure to the media (e.g., those more closely affected by the Boston Marathon bombings watched more news coverage, resulting in “a higher acute stress response” (Meridian University, para. 20). Collective trauma refers to the widespread prevalence of individual trauma experiences like confusion, difficulty concentrating, anger, insomnia, nightmares and flashbacks, guilt, shame, and/or fatigue (Meridian University). Therefore, this study takes into account trauma both on the individual and community scale.

These nuanced understandings of trauma have led to an interdisciplinary push for the implementation of a practice known as trauma-informed care. According to the Trauma-Informed Care Implementation Resource Center, an informational resource developed by the U.S.-based nonprofit health policy organization the Center for Health Care Services (CHCS), the key to implementing trauma-informed care is “shift [ing] the focus from ‘What’s wrong with you?’ to ‘What happened to you?’” (“What is Trauma-Informed Care?,” para. 1, emphasis in original). By viewing an individual as a “whole person” with a complex past, practitioners can better begin to care and provide individualized services for those suffering from lingering trauma (Arledge & Wolfson, 2001, p. 91). The Substance Abuse and Mental Health Services branch of the U.S. Department of Health and Human Services (Huang et al., 2014) has developed six core principles of care to help practitioners apply trauma-informed care to their respective areas (see Table 1). These principles promote respect for safety, transparency, peer support, collaboration and mutuality, empowerment and voice, and cultural sensitivity, which guide best practices.

Table 1
Six Core Principles for Trauma-Informed Care (Huang et al., 2014, p. 11).

Safety	Throughout the organization, both the professionals and clients should feel physically and psychologically safe.
Trustworthiness and Transparency	Decisions are made with transparency, and with the goal of building and maintaining trust.
Peer Support	Individuals with unique lived experiences are integrated into the organization and viewed as integral to producing equitable services.
Collaboration and Mutuality	Power differences are acknowledged to support shared decision-making.
Empowerment, Voice, and Choice	Individual strengths are recognized, built on, and validated.
Cultural, Historical, and Gender Issues	Biases and stereotypes that are based upon race, ethnicity, sexual orientation, age and/or geography, as well as historical trauma, are recognized and addressed.

2.2. Approaches to trauma-informed practices

Trauma-informed practices within different fields “have been authored in relative isolation from each other” (Foot, 2020, p. 1129). As such, the application of what it means to be trauma-informed can and has looked different in diverse fields. For example, Dr. Jeffrey Millstein (2020) encourages fellow physicians to ask the simple question: “Have you experienced anything that makes seeing a doctor difficult or scary for you?” (p. 1999) to discover why some patients might be averse to receiving medical care. Millstein further describes trauma-informed care as a bridge that is built between the physician and patient that can create a sense of safety that the patient can cross whenever they are ready (p. 2200). While the medical field is currently the most closely associated with trauma-informed practices, there are still fields in which the knowledge of how trauma can pervade a person’s life has yet to be integrated into everyday practices—one such field being that of public relations. This is important because as Foot (2020) argued, trauma-informed approaches are a way “for the ideals of each profession to be actualized” (p. 1136).

Within the field of communication, journalism has made the greatest strides to apply trauma-informed practices and considerations to their work. This has included considering the impact of trauma on journalists (e.g., Beam & Spratt, 2009; Dworznik, 2006; Newman et al., 2003), as well as the role in journalism education in helping future journalists navigate the emotional and ethical challenges associated with covering traumatic events (e.g., Amend et al., 2012; Dworznik & Grubb, 2007). One important area of trauma-informed practice for journalists is in interviewing sources, with a focus on not causing vulnerable subjects additional harm (Newman et al., 2023). This also involves considering journalist’s own emotional management when interviewing sources (Hopper & Huxford, 2017). Katz & Haldar (2016) also argued for the importance of trauma-informed practice for law students with focuses on “identifying trauma, adjusting the attorney-client relationship, adapting litigation strategy, and preventing vicarious trauma” (pp. 359–360). Foot (2020) even argued that legal advocates and journalists can learn from each other in terms of trauma-informed professional practices, specifically in the context of human trafficking survivors. Especially in fields focused on developing relationships, whether that be between attorney and client, journalist and source, or organizations and publics, professionals who are not trauma-informed can inadvertently cause harm to others who have experienced trauma (Foot, 2020).

While discrete professional fields, journalism and law do have significant overlap with many of the functions of strategic communication that can be seen in the arguments that Foot makes. Listening is one of the primary areas in which Foot argues for an overlap between lawyers and journalists when it comes to trauma-informed practices. This allows for the potential of engaging in dialogue, which is a key function of strategic communication (e.g., Kent & Taylor, 2002). Dialogue can serve a

particularly important role in strategic communication for emotional and traumatic topics such as sexual assault (Madden, 2018). This looks like allowing those who have experienced trauma voice and agency in telling their story, both in content of what is told and timing and context of when it is told.

2.3. Listening

Listening is necessary to foster personal and professional relationships, develop policy, build communication programs that meet constituent needs, and manage understanding among diverse publics (Wolvin, 2010). It is a complex process and phenomenological experience involving auditory, visual, and psychological functions, as well as memory, perceptual, and linguistic filters to co-construct meaning (Parks, 2019; Wolvin, 2010). Authentic listening involves listening and responding with honesty, avoiding jumping to conclusions, remaining attentive to equality, being actively involved in the listening process, and considering “where the speaker is coming from” (Floyd, 2010, pp. 130–131). Effective listening requires listening to oneself and the other, critical thinking, the inclusion of multiple perspectives, and making of choices that will affect the dialogic process (Parks, 2019, p. 4). In her study of ethical listening, Parks (2019) found that the process of listening must embody individuals working together, being vulnerable and reciprocal, and engaging with listening partners in ways that enable speakers to feel attended to via eye contact, quietness, encouraging acts.

Scholars have described listening with consideration for trauma as a “cultural and discursive process” (Cheddie, 2007, p. 80) that involves displaying presence, sympathy, acceptance, understanding, or silence via verbal and nonverbal cues (Bavelas et al., 2000; Cheddie, 2007; Harber, 2023; Kluger & Itzchakov, 2022). As a result of listening, those disclosing their trauma may experience reduced stress, gain emotional courage, understand issues from multiple perspectives, or uncover new meanings (Bavelas et al., 2000; Harber, 2023; Itzchakov & Reis, 2021; Itzchakov et al., 2017). Describing listening as a method of trauma research, Ratnam (2019) underscored how stories of trauma can be particularly understood via empathetic listening to both verbal and nonverbal elements of individuals’ experiences and stories. She described listening by observing participants’ practices in the home and nonverbal cues such as shrugging shoulders, shedding of tears, or smiles (Ratnam, 2019).

Engaging in authentic and responsive listening to trauma, however, can be harmful to or jeopardize listeners’ own belief systems (Harber, 2023). Journalists, social workers, counselors, and jury members who have listened to trauma have reported both psychological and physical problems including burnout, difficulty sleeping, depression, or gastrointestinal distress (Aiken, 1996; Kaplan & Winget, 1992; Lewis & Manusov, 2009). “Compassion fatigue” is a state of exhaustion and disfunction resulting from prolonged listening to trauma (Figley, 1995, p. 23). Compassion fatigue may be particularly compounded when the listener is highly empathetic, has experienced their own forms of trauma, or has unresolved trauma (Figley, 1995, as cited in Lewis & Manusov, 2009). Drawing upon extant listening research, scholars have recently begun to build upon the concept of ‘organizational listening,’ as a model for effectively implementing listening competencies, practices, and programs.

2.4. Organizational listening

Listening in public communication and public relations contexts has been approached historically from an organizational listening perspective (e.g., Macnamara, 2016a, 2016b, 2018). Such a perspective holds that listening is an often mediated and asynchronous management process centered on how an organization develops and implements listening policies to collect and analyze stakeholder feedback (Falconi, 2016; Macnamara, 2018). Effective organizational listening may best be facilitated via an ‘architecture of listening’ including a culture that

supports listening, policies that foster listening, open and interactive systems of communication, technologies and monitoring tools for listening, and proper resources, employees, space, and knowledge to listen (Macnamara, 2016a, 2016b, 2018). Studying a New York-based global public relations agency, Place (2019) found that the agency’s architecture of listening featured structural accommodations to encourage listening, programs and policies to support listening, a dedicated environmental scanning and listening work team, and technologies and tools to facilitate listening. Listening was utilized by practitioners to become enculturated into agency life, develop relationships with colleagues or clients, and identify cultural or industry trends.

Amidst an architecture of listening, training and education are essential for facilitating utilization of feedback tools, listening research techniques, listening via digital media, and integration of psychological, sociological, and anthropological theory and practice (Macnamara, 2016b). Methods of listening comprising an architecture should emphasize taking turns when listening, active engagement in listening via meetings, summits, boards, or juries that ensure representation from all publics (Macnamara, 2016b). Organizational listening is particularly dependent upon human resources, time, skills, and structures to listen. Delegated listening professionals must be able to recognize who must be listened to, be fluent in qualitative and technical skills, and be adept at collection, review, analysis and reporting of feedback (Claro, 2022; Macnamara, 2018). Ultimately, organizational listening should particularly respect others’ right to speak, acknowledge others’ views, pay attention to others, interpret what others say fairly, work to understand others’ views, consider what others say, and respond appropriately (Macnamara, 2018, pp. 6–7). Such organizational listening concepts align with trauma-informed concepts of transparency, safety, support, collaboration, empowerment and voice, and cultural understanding (Huang et al., 2014).

Unfortunately, scholarship has found that organizational listening is undervalued and under-practiced in the public communication fields, only accounting for about 20% of professional work and focusing heavily on optimal message distribution (Macnamara, 2016a, 2016b). Research has indicated, however, that listening in strategic communication must occur to better acknowledge marginalized voices, listen across cultures and communities, and tap into local knowledge (Macnamara, 2016a, 2016b; Sirolli, 2012). Organizational listening must acknowledge and attend to emotional, angry, and diverse sentiments with sensitivity, with regard for power relations between organizations, stakeholders, and publics (Macnamara, 2016b).

Recent ethical and intersectional approaches to organizational listening, particularly in the wake of social movements such as #BlackLivesMatter, #DontSayGay and #StopAAPI hate, have emerged to complement such scholarship. They underscore the importance of listening to foster trust, adequate time spent engaging in listening, gaining buy-in for listening among organization’s senior leadership, reflexivity of personal identity and bias when listening, and acknowledgement of fluid, overlapping, or contested identities of diverse publics while listening, respectively (Neill & Bowen, 2021; Place, 2022). Organizational listening practices for diversity, equity, and inclusion have grown to harness architecture of listening tools of surveys and protocols, meetings, organizational employee resource groups, designated organizational listeners, and grassroots approaches to listen to diverse publics (Place, 2023). Listening in alignment with intersectional and corporate responsibility to race (CRR) approaches may provide particularly effective means for supporting authentic, equitable, and social justice focused organizational listening (Logan, 2021; Place, 2022, 2023).

Whereas extant research in disciplines outside of public relations has addressed the concepts of listening and trauma, public relations-focused research has not yet explored trauma-informed listening. Moreover, extant organizational listening models, such as the architecture of listening (Macnamara, 2016b, 2018), can benefit from additional

theoretical and practical development for public relations contexts. Drawing upon literature regarding trauma-informed approaches, listening, and public relations, the purpose of this study was to explore how public relations professionals employ trauma-informed practices as they listen. One research question guided this study:

RQ. How, if at all, do strategic communications professionals engage in trauma-informed listening?

3. Method

A qualitative method incorporating in-depth interviews was chosen for this exploratory study of trauma-informed listening because it enables for the collection of thick description, the development of a holistic understanding of the phenomenon under examination, and the understanding of meaning-making with regard to particular contexts. This paper draws upon data from a larger study regarding meaning-making and practices regarding listening in strategic communication and public relations. The sampling, recruitment, procedural, and analysis methods informing this paper are detailed below.

3.1. Sampling and recruitment

After gaining institutional review board (IRB) approval for the study, professionals responsible for an organization's strategic communication and public relations efforts under the definition of "building mutually beneficial relationships between organizations and their publics" (PRSA.org) were recruited using purposive and snowball sampling methods. First, a purposive sampling method was used to recruit a diverse sample of participants in terms of expertise, leadership level, and geographic reach. Professionals received an email recruitment letter inviting them to participate in the study. Later, interviewees were recruited using a snowball sampling method, as participants from the purposive sampling method recommended individuals to participate in the study. Those individuals then received a recruitment e-mail to participate.

The resulting sample of participants informing this paper included 54—New England (33), Mid Atlantic (8), Southern (6), Midwestern (4), and West Coast (3)—strategic communication professionals and executives representing nonprofit (32), agency (11), corporate (5), education (3) and governmental (3) organization types, particularly those that serve marginalized communities. Regarding gender identity, 34 participants identified as women, 18 participants identified as men, one participant identified as nonbinary, and one participant identified as transgender. Regarding sexual identity, six individuals identified as lesbian or gay whereas 48 identified as heterosexual or straight. In terms of race and ethnicity, 10 participants identified as Black, four participants identified as Latinx, one participant identified as Asian, one participant identified as Arab American, and 38 of the participants identified as White. Most participants (30) represented the most senior or executive-level position responsible for strategic communication or public relations within their organization, such as Director of Communications, Director of Advocacy, National Director, Director of Marketing and Public Relations, or Executive Director. The remainder represented mid-level communications professionals who oversaw the public relations or communications functions and held the titles of Public Relations Specialist, Regional Communications Manager, or Community Engagement Specialist.

3.2. Procedure

Interviews were conducted by the principal investigator between February and May 2021 and guided by a 16-question interview protocol featuring a variety of rapport-building, open-ended questions, probes, and follow-up questions in a pre-determined order (Rubin & Rubin, 2005). Sample rapport-building questions pertaining to this paper included, "What do you like best about your role as a communications

professional at [Organization]?" and "When you think of 'listening,' what does that term mean to you?" Sample open-ended questions addressing trauma-informed listening included, "Describe ways in which you have listened with consideration for trauma," "What is an example of a time when you engaged in trauma-informed listening?" and "Please tell me about any training or support have you received for trauma-informed listening?" Additionally, probes and follow-up questions, such as "Why?" "How?" or "Could you elaborate on..." were used to encourage participants to elaborate upon their responses and offer examples. All interviews were audiotaped, conducted via Zoom due to the COVID pandemic, and ranged from approximately 40 min to 90 min in length.

3.3. Analysis

Interviews were transcribed professionally, analyzed, and coded word-for-word among the principal investigator and two additional researchers. Using a thematic analysis approach (Boyatzis, 1998) the transcripts were read and re-read multiple times for themes related to listening and trauma. The researchers met periodically during the coding and analysis process to discuss the emerging themes and their relationship to the research question, to achieve consensus about themes, and to ultimately organize the themes in order as appears in the paper.

Annotations and memos were also written in comment boxes among the transcripts to identify connecting themes or discuss contextual features of the themes (Corbin & Strauss, 2008; Miles & Huberman, 1994). Additionally, the three researchers remained mindful of how their own identities, lived experiences, and preconceptions could affect how the data was collected, analyzed, and interpreted. Lastly, member checks (e.g., Lincoln & Guba, 1985) were used to confirm or check the meanings of particular responses. Some quotations were edited in the manuscript to remove verbal placeholders, but original meanings of passages were maintained.

4. Findings

Five themes emerged during analysis regarding how public relations professionals engage in trauma-informed listening. Not all participants readily identified or practiced trauma-informed listening. However, participants who did so described how trauma-informed listening was accomplished with regard for (1) respect and sensitivity, (2) listening out for invisible or unheard trauma, (3) consideration of power distance, (4) understanding that trauma can be inflicted by communications work, and with (5) consideration for compassion fatigue. Evidence for each of these themes is offered below.

4.1. Listening with respect and sensitivity to trauma

Participants' discussions of trauma-informed listening commonly utilized the words "respect" and "sensitivity" to "meet individuals where they are." A communications manager for a New England-based immigration services nonprofit explained, for example:

It's being very respectful with how you're listening and engaging, right?. You're speaking to our clients who have gone through their share of trauma. So we just want to be extra sensitive to them...being sensitive to what our clients have experienced, just like the cultural differences and the cultural sensitivities.

Quite similarly, a communications research director on behalf of an East Coast-based regional commerce authority explicitly identified her listening as "trauma-informed" with sensitivity for individuals. She described her work listening, particularly to marginalized publics her organization serves, as being "supersensitive trying to listen to people's stories.... I was doing the same things that the social workers were doing – trying to learn from the social workers how to ask questions, how to

speak, how to hear someone's story."

Many participants recounted times when their professional roles called for them to engage in trauma-informed listening with sensitivity and respect during or in response to traumatic events. The public relations and marketing director for a regional disaster relief nonprofit described her use of trauma-informed listening as "being there" for those experiencing trauma. Likewise, a communications director for an East Coast-based religious charity organization reflected upon their personal first-hand experiences with trauma that helped them to then listen with sensitivity and respectfully cover events in the wake of current social unrest. They explained,

I was able to spend time with [individuals in trauma] and try to portray one of the darkest things in our own history, that is domestic violence between our own communities...It was a personal thing to learn more and see how well I was able to portray it with dignity, because for me, that was what was important. And it's the same with everything that I do at [organization]. [Acknowledges events of racism and violence in Detroit]. But how are we going to represent that? And I wouldn't know that if it wasn't because I spent a lot of my life talking to people...and letting them talk.

Vignettes above speak to ways that participants listened with respect and sensitivity to achieve a sense of safety, trustworthiness, and peer support among their publics (Huang et al., 2014).

4.2. Listening for unspoken or invisible trauma

In addition to respect and sensitivity, participants also described how trauma-informed listening involved listening out for unspoken or invisible trauma. A community services director for a regional emergency aid nonprofit explained how trauma-informed listening involved discerning that trauma may have occurred because many people do not realize or do not want to admit that something was traumatic. As such, trauma-informed listening involves "recognizing that trauma looks different. There's no one definition for trauma. So if we know that there's trauma, how do we come alongside someone and say, trust me? Hopefully with a rapport we build with them..."

Likewise, a midwestern public relations agency account supervisor acknowledged the importance of trauma-informed listening for unspoken trauma for internal audiences, particularly in the wake of the murders of George Floyd and Ahmaud Arbery. He shared a personal account of how his agency failed to listen out for trauma among Black employees – and instead, gave them a survey to complete (which actually re-traumatized many employees). He lamented, "I get what you guys were trying to do, but maybe approach it like this, like maybe *listen*, maybe look, get the employee resource group." The employee resource group was mentioned as an important resource because it allowed them to "grow...mentally, emotionally and professionally" rather than feeling like a "test" and "more work" for potentially traumatized employees.

Some participants noted how trauma-informed listening among marginalized groups required listening out for the unspoken forms of shared trauma, while simultaneously listening out for the unspoken forms of nuanced identity- or lived experience-based trauma. The director of a New England-based LGBTQ+ nonprofit shared an example of such a trauma-informed listening balance. First, he explained trauma-informed listening to acknowledge:

...that the one shared experience that every queer person has is the fear that their parents will not love them. It's the only true shared experience that every single person has, because it doesn't matter what your parents are like. You have that moment as a kid where you fear that your parents are going to stop loving you.

However, the director acknowledged the importance of listening across multiple lines of oppression, discrimination, or advantage. He lamented that for every White, cisgender gay man there is a guilt at the taking away of voices of 40 trans individuals or 40 poor individuals.

"Well, what about all the rest of the community? They're not getting represented at all. And so there's this like intercommunity guilt being created that is terrible... It's just it's we're all fighting for scraps instead of recognizing we're living off of scraps," he shared. Trauma-informed listening in public relations, therefore, involves complex attention to verbal and nonverbal cues regarding both publics' identities and lived experiences (Bavelas et al., 2000; Ratnam, 2019) and cultural, historical and gender dynamics (Huang et al., 2014). Amidst these complexities, participants also described how trauma-informed listening was attentive to power relations.

4.3. Listening for power distance amidst trauma

Trauma-informed listening was connected intrinsically to power, particularly an understanding of power difference or power distance (e.g., Hofstede, 1985). A senior director of communications for a Southern-based international nonprofit spoke of her trauma-informed listening in global regions (e.g., Africa and Middle East) where women have experienced gender-based violence and trauma and often lack opportunities for education, work, or travel. She spoke to the importance of ensuring appropriate power- and gender-based listening dynamics to foster trust. She explained:

I'm really excited about having more women involved in our work on the ground because I think when I've been there...it's an *event*. It's overwhelming and intimidating. And I think that having fewer people, having someone who can really spend time...to really get to know people and have them feel comfortable. I think when you have people that are in suits, men, that...we've had to work really hard to say, 'We want her to go by herself. She's fine to go by herself. She can be safe by herself. She doesn't need this army of people.' That allowing for really open, even intimate conversation, that's real. And there's not that sort of pressure.

Similarly, a press secretary for a U.S. Senator underscored the importance of trauma-informed listening with consideration for power-distance, particularly among underrepresented publics. The press secretary shared an account of a listening session about COVID childcare held with the White U.S. senator at a daycare in a hard-hit, low-income, African American community. They showed up in suits with four camera operators, not considering how "frazzling and frightening" the experience may have been for the children, daycare workers, and parents. He described their *lack* of trauma-informed listening with:

It's something that you or I would take for granted....these are hard hit communities. They've lost loved ones. You know, it's really been difficult on them. And I'm showing up at a daycare center in the heart of [neighborhood] with, you know, four television cameras, a newspaper reporter, a United States senator and the mayor of [city] and all the people... I realized for them, it's probably like, it's going to be overwhelming...We have to be sympathetic to that and not take advantage of [constituents].

Ultimately, participants suggested that trauma-informed listening with regard to power distance—particularly to individuals traumatized by racism—involved recognizing it's not about those in power "giving voice," but authentically and organically listening to publics. A communications coordinator for a regional immigrant services nonprofit explained how he listens with underrepresented communities with the understanding that,

...these are not people that don't have a voice. We're not *giving* anyone a voice. They have a voice. They just never had a platform because they were, you know, hindered by, you know, White supremacy and xenophobia. Racism. We're just a bridge with just a tool that they can use so they can use their voice using our resources. I think just listening is important, then we can restructure and reframe

the language that we use when we speak about this. Then [racism] wouldn't happen if we were listening to our community.

4.4. Recognizing the trauma inflicted by communications work

Like recognition of power, participants acknowledged how their trauma-informed listening involves consideration for how they could potentially traumatize or re-traumatize target publics or constituents. The communications director for a regional food bank in the Northeast explained her trauma-informed approach to listening and consideration of re-traumatization. She shared:

When people tell their stories, sometimes they might be in the middle of food insecurity or actual hunger...But there is so much trauma around that story that after you've pulled it or extracted it from them later on, they are like "What did you do? I'm now vulnerable." ... It's like we can't re-traumatize people because it is very, very traumatizing. And then we get the story and then we go, 'Oh, it's on our website' and we share it on social media. We connect them with journalists. But when they think about or when that story is out, you know, they can't take it back. I try to be mindful of that and ask people, 'Is this what you want? Do you want to share your story?'

Quite similarly, the director of public relations for a national nonprofit addressing youth hunger shared how she considers social and emotional welfare in addition to where and how listening occurs. Ensuring that constituents are physically and emotionally comfortable in listening environments can minimize re-traumatization. She noted, "We're talking about kids who are healing from trauma. Sometimes when that happens, the garden is a really great place to bring kids who are dealing with issues... So we would bring them to the garden and do hands-on things with them in the dirt... We really wanted to make sure we were doing that in a way that didn't negatively impact these kids."

4.5. Listening and compassion fatigue

Listening as a public relations professional often involves being the go-between or front line of listening between an organization and its publics. Therefore, several participants described how trauma-informed listening can result in burnout or "compassion fatigue" (Figley, 1995; Lewis & Manusov, 2009) with considerable emotional and physical consequences. The director for a regional social justice nonprofit in the Northeast described how she and her communications staff experienced compassion fatigue via their intensive listening work in the communities they serve. She stated,

You can't just say to somebody, 'take a time out' because they live there or they hang up on us for services and housing subsidies. And of course, vicarious trauma for our staff is hard. Because we hear stories, they experience stuff, they have stuff aimed at them. And it takes its toll. And I think one of the areas that we've got to pay close attention to because we have some staff who have been doing this for a long, long time, is not just regular fatigue, but compassion fatigue.

To engage in trauma-informed listening on the job, yet combat compassion fatigue, several practitioners shared their survival strategies. The public relations director for an East Coast civil liberties organization explained how institutional support of taking time off and setting boundaries ensured healthy work habits. She said,

I think institutional support is really important. So our organization has a trauma policy. So when the staff are feeling, for whatever reason, whatever issue, feeling the heat and that trauma, we are allowed to take time off. So I think personally, I try to set boundaries. But we get a lot of hate directed at us as well - as the other part of listening that we end up doing. And my colleague is a Black man, I'm a White woman. I get a lot of gendered and racist things thrown at us, so I set boundaries around unplugging...

Likewise, the communications director for an East Coast immigrant services nonprofit shared experiences of trauma-informed listening with those who had experienced suffering or violence. Citing advice from mentors and drawing up on career experience, they stated:

We work every day with asylum cases and all the spectrum of, like immigrant families that were coming to this country and trying to get asylum. And I think that it is definitely another learning opportunity for me to see how hard it is and how complicated it is for you to understand where you need to pull away. And give some time for yourself...you cannot help everyone if you don't help yourself. And I've understood that after years of working in trauma.

5. Discussion

Previous research has called for increased integration of psychological, sociological, and anthropological theory and practice to enhance listening for public and strategic communication (Macnamara, 2016b). This qualitative study of 54 public relations professionals built upon extant listening, public relations, psychology, and sociology research to explore trauma-informed listening in public relations to answer this call. Findings suggest that trauma-informed listening in public relations involves considerable amounts of sensitivity, understanding of power distance (Hofstede, 1985), reflection on the potential for re-traumatization of individuals, and acknowledgement of one's own personal listening fatigue. Participants' passages and themes complemented the principles of trauma-informed care with mentions of listening to promote transparency, safety, peer support, collaboration, empowerment, and cultural understanding (Huang et al., 2014). Because trauma among publics may be unspoken or invisible to practitioners, an emphasis on listening for both verbal and nonverbal cues during trauma-informed engagement is key (Bavelas et al., 2000; Ratnam, 2019). In particular, participants cited ways in which they listened with sensitivity and nuance to build safe, trusting, and empowering environments. Moreover, public relations work is increasingly occurring on a global scale, necessitating attentive listening with regard to intersections of geography, race, ethnicity, gender, and power difference. Trauma-informed listening may offer a particularly strong pathway for public relations practices to better consider publics as "whole people" with complex lives who experience a wide range of fluid, overlapping, and evolving avowed and ascribed identities (Arledge & Wolfson, 2001, p. 91).

The notion of trauma-informed care as a "bridge" (Foot, 2020; Millstein, 2020) is particularly important to guide trauma-informed listening for public relations. As the coordinator of regional immigrant services noted, "we are just a bridge" who listen and engage with publics with care. Trauma-informed listening for public relations, therefore, can be a bridge function to facilitate listening to diverse stakeholder groups, foster dialogic connections between organizations and publics who have experienced trauma (i.e., Bridgland et al., 2021; Lonsdorf, 2022), and support boundary-spanning work to identify instances of trauma among publics. Foot (2020) suggested that trauma-informed care works as a bridge where "ideals for each profession can be actualized" (p. 1136). Integrating concepts of trauma-informed care in public relations listening contexts offers a ripe opportunity for further combined implementation of the profession's ideals of listening, ethics, fairness, patience, character, and public interest (PRSA Code of Ethics; The Page Principles).

5.1. Enacting trauma-informed listening: a way forward

The authors propose the integration of trauma-informed listening competencies into the 'architecture of listening' framework (Macnamara, 2016a, 2016b, 2018). Table 2 synthesizes knowledge gained from participants about trauma-informed listening practices, how this aligns with trauma-informed principles (Huang et al., 2014), and ways that

Table 2

Trauma-Informed Listening for Public Relations.

Trauma-Informed Listening by PR Practitioners	Trauma-Informed Concepts (Huang et al., 2014)	Principles of Active Organizational Listening (Macnamara, 2016a, 2016b, 2018)	Ideas for Enacting Trauma-Informed Organizational Listening
Listening with respect and sensitivity to trauma	Safety; Trustworthiness and transparency; Empowerment, voice, and choice	Create an organizational culture of listening Establish policies that specify and require listening in an organization Implement technologies that aid listening Develop skills/competencies for large-scale organizational listening	Learn more about trauma-informed approaches. Trauma Informed Oregon is a great resource to start. (https://traumainformedoregon.org/) Consider the emotional and physical safety of participants when listening to them (TIO, 2014). Reduce uncertainty. Knowing when opportunities for internal and external stakeholders to be listened to is important. It should be routine (Imad, 2022). Have different types of methods for listening. This can include surveys, town halls, and/or individual interviews. Give people a choice of how to participate, including online and anonymous options. Consult all relevant stakeholders (internal and external), including historically marginalized groups. Consider who is planning and implementing listening sessions, and how that might impact what people feel comfortable sharing. Practice ongoing consent for using stories and testimonials from stakeholders. Offer opportunities for people to regularly re-consent (Leahy, 2022). Listen not only to those who are the most vocal, but also for the silences. Gilligan's (2015) Listening Guide provides helpful questions: Who has a voice? Whose voices are heard? Which voices are heard? What do silences represent? What is considered to be silence? (Harel-Shaleve & Daphna-Tekoah, 2018). Consult resources on trauma-informed interviewing for media professionals (Brayne, 2007). Stewardship principles are important. What feedback loops are in place? Be transparent in how information is being used and what action is being taken. How will that be communicated? (Madden et al., 2022) Trauma-informed listening must be action-oriented. Truly listening to people, especially when harm may have been done, involves being willing to change (Becker-Blease, 2017). Development of employee resource groups or affinity groups as places for peer support and listening (Place, 2022, 2023).
Understanding that trauma can be inflicted by communications work Listening for invisible or unheard trauma	Cultural, historical, and gender issues; Collaboration and mutuality	Address the politics of listening	
Consideration of power distance	Collaboration and mutuality; Empowerment, voice, and choice; Peer support; Trustworthiness and transparency	Design systems that are open and interactive Have channels for articulation of what is said to an organization to policy making and decision making	
Consideration of compassion fatigue	Safety; Peer Support; Cultural, historical, and gender issues	Allocate resources to do the work of listening	Consider who people turn to for support. Ensure the burden of listening is not falling disproportionately on certain groups of people, particularly women and people of color (Madden & Del Rosso, 2021). Develop a trauma policy that allows people to take time off and set boundaries if/when feeling compassion fatigue.

existing 'architecture of listening' facets tangibly support these practices.

5.1.1. *Listening with sensitivity and respect to trauma*

We view this aspect of trauma-informed listening aligned with trauma-informed principles of safety, trustworthiness and transparency, and empowerment, voice, and choice. People must feel safe and empowered to tell their stories, and there must be trust and transparency for how that information will be used. Trauma-informed organizational listening starts with creating an organizational culture of listening grounded in those principles. In practice, therefore, the public relations field would benefit from offering onboarding opportunities that identify trauma-informed listening and interpersonal listening competencies among potential job candidates. Training and resources from places such as Trauma Informed Oregon (traumainformedoregon.org) also offer a common language and context to create a trauma-informed organizational listening culture grounded in specific skills and competencies. Creation of policies that specify and require organizational listening help to reduce uncertainty and provide consistency for both internal and external publics. Giving people a choice of how they prefer to be listened to is also important. Have different types of methods for listening including surveys, town halls, and/or individual interviews. Give people a choice of how to participate, including online and anonymous options.

5.1.2. *Recognizing potential harm and listening for silences*

We see two of the themes that emerged from our interview data as directly related to and informative for understanding what addressing

the politics of listening means. The trauma-informed principles of cultural, historical, and gender issues, as well as collaboration and mutuality, are instructive for guiding our thinking here. Our work as public relations professionals always has the potential to inflict trauma, but risk can be minimized by consulting all relevant stakeholders (internal and external), including historically marginalized groups. Listening must occur during planning, implementation, and evaluation, and who is doing the listening must be considered in terms of how comfortable people feel sharing. It is also vital to practice ongoing consent as part of trauma-informed organizational listening. Offer opportunities for people to regularly re-consent to stories and testimonials being used (Leahy, 2022). Resources from places such as the Dart Centre, while targeted towards journalists, also offer public relations professionals guidance on how to conduct trauma-informed interviewing (Brayne, 2007). And finally, listening for silences is a key trauma-informed listening practices. Gilligan's (2015) Listening Guide provides helpful questions for practitioners to consider: Who has a voice? Whose voices are heard? Which voices are heard? and What do silences represent? Harel-Shaleve and Daphna-Tekoah (2018) also added the question, What is considered to be silence?

5.1.3. *Consideration of power distance*

We believe that consideration of power distance must be guided by several trauma-informed principles, including collaboration and mutuality; empowerment, voice, and choice; peer support; and trustworthiness and transparency. We believe that architecture of listening guidance of designing systems that are open and interactive, as well as having channels for articulation of what is said to an organization to

policy making and decision making, can support this. Trauma-informed listening must be action-oriented or it just serves to reinforce the status quo, which may be causing harm. Truly listening to people, especially when harm may have occurred, involves a willingness to change (Becker-Blease, 2017). Stewardship principles can also be important for trauma-informed listening. What feedback loops are in place? It must be made transparent how information is being used and what action is being taken. Public relations can play an important role in communicating this (Madden et al., 2022). Another way to consider power distance within trauma-informed listening is to include opportunities for peer support and listening. Development of employee resource groups or affinity groups as places for peer support and listening are a good practice (Place, 2022, 2023).

5.1.4. Consideration of compassion fatigue

Listening, especially when being attuned to trauma, can result in burnout and compassion fatigue (Figley, 1995; Lewis & Manusov, 2009). The trauma-informed principles of safety, peer support, and cultural, historical, and gender issues are instructive. As suggested by the architecture of listening, organizations must allocate resources to do the work of listening. As such, public relations would benefit from training that draws upon concepts from counseling, medical, sociological, and psychological approaches to enhance practitioners' trauma-informed listening competencies. Such competencies would help prepare professionals to manage the emotionally and physically taxing forms of listening work that occur in public relations (e.g., Amend et al., 2012; Dworznik & Grubb, 2007). Organizations must also consider who people turn to for support and ensure the burden of listening is not falling disproportionately on certain groups of people, particularly women and people of color (Madden & Del Rosso, 2021).

Improved partnerships across organizational functions, such as communications and human resources, may provide fruitful paths for future development of organizational architectures of listening that are trauma-informed. For example, the development of a trauma policy that allows people to take time off and set boundaries if/when they are feeling compassion fatigue can help create a trauma-informed organizational listening culture that mitigates burnout.

5.2. Limitations and directions for future research

Because the global pandemic and social unrest were very much on the minds of the interview participants, it is possible that some participants themselves were experiencing trauma. The interviews may have revealed very different vignettes and insights if conducted outside of a time of collective trauma. The sampling method utilized for the project represents an additional limitation. Interview participants were primarily non-profit public relations professionals identified based on convenience and through word-of-mouth snowball sampling method. It is possible that interview participants suggested colleagues with similar practices or perspectives to their own. Future research projects could, therefore, have a more representative sample of interview participants and explore trauma-informed listening across more diverse business or corporate sectors. Likewise, longitudinal studies of trauma-informed listening in public relations would be beneficial to understand effects of growing offerings of trauma-informed training in the profession or how trauma-informed perspectives or approaches change over time. Lastly, future research may wish to explore how teaching of trauma-informed care and trauma-informed listening skills in public relations courses can enhance students' professional competencies.

6. Conclusion

Trauma is complex, and it is not the purpose of this project to overstep the bounds of engaging with trauma in a therapeutic context. At its core, a trauma-informed approach to listening in public relations is simply rooted in considering the humanity of both the communicators

and their publics. As this study found, this may be more clearly seen in nonprofit and governmental contexts given the nature of the roles of these organizations in society. The authors believe there is a huge opportunity in public relations to further explore how corporate and agency environments may also engage in trauma-informed listening. These competencies will be increasingly demanded by employee publics, diverse clients, and diverse audiences grappling with the realities of life in the wake of social unrest, COVID 19, and global political strife. The connections between the architecture of listening, trauma-informed practices, and the lived experiences of communicators create an important springboard for ongoing work in this area.

Declaration of Competing Interest

We declare that the manuscript does not pose a conflict of interest.

Acknowledgements

The authors gratefully acknowledge funding from The Arthur W. Page Center for Integrity in Public Communication.

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